

Attempted realistic observations on 2021

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There was some research earlier this year that I thought was apt for the pandemic, showing that realism is key to being happy, not optimism or pessimism.

In particular, it is notable that ‘unrealistic optimism – the tendency to overestimate the likelihood that good things will happen and underestimate the likelihood that bad things will happen – is one of the most pervasive human traits’. However, the problem is that the disappointment that comes from misplaced optimism causes substantially more distress, psychologically, than being pessimistic: ‘the pain of an unexpected loss of £5 is twice as strong as the joy of an unexpected gain of £5’.

Everyone wants 2021 to look better than 2020. Hurrah! There’s a vaccine! Yes, there is. But as we think ahead to Autumn 2021, what does it *actually* look like *at the moment*, other than our *hope* that things will be better? How can we avoid this human trait of tending towards optimism and finding ourselves brutally disappointed. It was not, as per the ridiculous claim of our Prime Minister, *ever* going to have been ‘over by Christmas’. So what does it look like to think about this neither pessimistically nor optimistically, but *realistically*?

1. There is currently no evidence of the vaccine programme having the hoped-for causal effect, because it has not been administered at the required scale. But we do *have* several promising vaccines. The outcome remains speculative, although I, as much as anybody (possibly even more

given how much danger I am in, personally, at present), hope that it will work. There is some cause to hope that the vaccine will protect the most vulnerable from death. It is not known whether it will prevent long Covid and the problematic secondary pathologies of the disease. Hospitals are getting better at treating Covid and preserving life, but not yet at stopping the secondary problems and long Covid.

2. We are currently in a worse situation, in epidemiological trajectory terms, to where we were last March – higher hospitalisations, much higher infection rates, higher death tolls. We are in a total national lockdown, anticipated for at least 3 months if government leaks are anything to go by. The virus continues to mutate, to its advantage, becoming more infectious. It is unclear what impact this will have upon the vaccination schedule that is optimistically mentioned in the document. Worldwide we are at almost 2m deaths.
3. The current schedule for vaccinations predicts that even those < 40 years old are not scheduled to be vaccinated until December 2021.
4. We do not know whether immunity conferred by the vaccine will *last* over a twelve-month period.
5. The vaccine will *not* bring me freedom. The advice for someone with as damaged an immune system as mine is to continue to take extreme care (shield), even after vaccination.
6. The Chief Medical Officer has said that there will need to be restrictions next winter.

My attempt at realistic hopes leads me to believe that this year will be another of near-total confinement for me and many like me. I hope that I will, though, be able to have some hospital tests to confirm whether I have autoimmune autonomic ganglionopathy. I hope that I will be vaccinated so that, if I am hospitalised as an emergency, I will not be at severe risk of dying. I hope that I will get to see my 100-year old Nan at some point this year and to see my one-year-old niece, after we all isolate beforehand. I hope I will get to see my mum and brother at some point. I hope that the vaccination programme will work and hospitalisations will drop off.